

# Altitude Sickness - Male (Child)

Algorithm 1187 | Child (0-17 years) | Gender at birth: Male

## Definition

- Safety-first assessment of illness after ascent to altitude, distinguishing acute mountain sickness from suspected high-altitude cerebral oedema (HACE), high-altitude pulmonary oedema (HAPE), and other emergencies.
- Child patient.
- Male.

## Initial assessment

1. [OPEN\_TEXT] What is the current and sleeping altitude, how quickly was it reached, when did symptoms begin relative to ascent, and is a lower safe altitude reachable now?
2. [YES\_NO] Is there confusion, unusual behaviour, severe drowsiness, inability to walk a straight line, loss of coordination, collapse, seizure, or reduced responsiveness?
3. [YES\_NO] Is there breathlessness at rest, marked breathing difficulty, chest congestion, worsening cough, frothy or bloody sputum, blue or grey colour, or a major fall in exercise ability?
4. [OPEN\_TEXT] Record age, pregnancy, heart or lung disease, sickle-cell disease, medicines or substances, possible injury, fever, carbon-monoxide exposure, and glucose if immediately available without delaying rescue.
5. [OPEN\_TEXT] What communication, trained companions, oxygen, shelter, weather protection, evacuation, and professional rescue resources are actually available? Do not delay descent or rescue to obtain equipment.
6. [CAREGIVER\_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

## Triage and redirect questions

### **1. Redirect → Carbon Monoxide Exposure**

Did symptoms begin after exposure to a heater, fire, generator, engine, or enclosed-space fumes rather than after ascent to high altitude?

A suspected toxic gas exposure belongs in the Carbon Monoxide Exposure pathway.

### **1. Disposition level 100**

Is there suspected HACE or HAPE: confusion or abnormal behaviour, inability to walk normally, loss of coordination, severe drowsiness, collapse, seizure or reduced responsiveness; or breathlessness at rest, respiratory distress, chest congestion, worsening cough, frothy or bloody sputum, blue/grey colour, or rapidly declining exercise ability?

HACE and HAPE are life-threatening. Descent is the definitive field action and oxygen is an important adjunct when genuinely available, but neither remote equipment nor medication replaces professional rescue and evacuation.

### **1. Disposition level 15**

Are symptoms limited to headache with nausea, dizziness, fatigue, or poor sleep after recent ascent, with normal breathing at rest, normal walking and thinking, no concerning alternative cause, and a reliable companion, communication, and descent plan?

Possible mild acute mountain sickness must not ascend higher. Worsening or failure to improve requires descent; unreliable observation, communication, or descent access lowers the threshold for rescue advice.

## Advice

### **Emergency HACE/HAPE precautions**

- Activate professional rescue immediately: call Qatar 999 if in Qatar, or the local emergency or mountain-rescue number when abroad. Begin assisted descent to a substantially lower safe altitude as soon as conditions permit; never leave the patient alone or make them walk if weak, breathless, confused, or uncoordinated. Minimize exertion, protect from cold, do not allow driving, and give supplemental oxygen only if available and operated by someone trained. Oxygen, a portable hyperbaric chamber, or previously prescribed expedition medicine must not delay descent or evacuation. If weather or terrain prevents descent, tell rescuers immediately and use only trained, available measures while awaiting rescue. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: worsening confusion, inability to walk, seizure, or reduced responsiveness; increasing breathlessness, blue/grey colour, or frothy/bloody cough.

### **Monitored care for possible mild altitude illness**

- Stop ascent and do not sleep higher. Rest with a reliable companion, avoid exertion, alcohol, sedatives, and opioids, and drink normally without forcing excess water. If symptoms worsen or do not clearly improve at the same altitude, descend with assistance and seek the governance-approved medical service. Do not rely on small recreational oxygen cans or start prescription altitude medicines unless they were prescribed for this patient with clear instructions.
- Call Back If: confusion, abnormal walking, severe drowsiness, collapse, or seizure; breathlessness at rest, worsening cough, chest congestion, blue/grey colour, or failure to improve.

## First aid

- Activate professional rescue immediately: call Qatar 999 if in Qatar, or the local emergency or mountain-rescue number when abroad. Begin assisted descent to a substantially lower safe altitude as soon as conditions permit; never leave the patient alone or make them walk if weak, breathless, confused, or uncoordinated. Minimize exertion, protect from cold, do not allow driving, and give supplemental oxygen only if available and operated by someone trained. Oxygen, a portable hyperbaric chamber, or previously prescribed expedition medicine must not delay descent or evacuation. If weather or terrain prevents descent, tell rescuers immediately and use only trained, available measures while awaiting rescue. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

## References

- NHS.UK, "Altitude sickness", <https://www.nhs.uk/conditions/altitude-sickness/> (page last reviewed 31 July 2023)
- US CDC Yellow Book 2026, "High-Altitude Travel and Altitude Illness", <https://www.cdc.gov/yellow-book/hcp/environmental-hazards-risks/high-altitude-travel-and-altitude-illness.html> (accessed 25 July 2026)
- Qatar Ministry of Public Health, "Healthcare Services in Qatar", <https://sportandhealth.moph.gov.qa/EN/faninfo/Pages/HealthcareServicesInQatar.aspx> (999 ambulance access; accessed 25 July 2026)

## Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from cited public sources; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect. Demographic variants contain only source-supported differences; identical thresholds are documented rather than fabricated. Local destinations and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

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